

ADVANCED NEUROLOGY & DIAGNOSTIC CENTER

DEPARTMENT OF CLINICAL NEUROSCIENCES & NEUROPHYSIOLOGY

NEUROLOGICAL CONSULTATION REPORT

Report ID: NEURO-2026-884920
Date: August 10, 2026

Patient Name:	Jonathan Doe	MRN / ID:	NEURO-884920
Age / Sex:	45 Yrs / Male (Right-Handed)	Referring Dr:	Dr. Marcus Vance, MD
Consult Date:	August 10, 2026	Attending Dr:	Dr. Elena Rostova, MD, FAAN

1. PATIENT METADATA & CLINICAL HISTORY

Chief Complaint: Intermittent right hand numbness, thumb/index paresthesias, and occasional localized tension headaches over 6 weeks.

History of Present Illness (HPI): A 45-year-old right-handed male presenting with a 6-week history of episodic distal right upper extremity paresthesias, predominantly affecting digits I–III. Symptoms intensify following computer use and occasionally wake him at night. Reports mild, non-throbbing holocranial headaches without red-flag symptoms (no nausea, photophobia, papilledema, or sudden thunderclap onset). Denies seizures, syncope, visual deficits, or focal weakness.

2. CLINICAL NEUROLOGICAL EXAMINATION

I. Mental Status & Cognition

- Alertness:** Alert, fully oriented x 4 (Person, Place, Time, Event).
- Language:** Fluent speech, intact naming, comprehension, and repetition.
- Memory:** Intact short/long-term memory (3/3 recall at 5 min).

II. Cranial Nerves (CN I – XII)

- CN II-III:** Visual fields full. Pupils equal, round, reactive (PERRL 3mm to 2mm).
- CN III, IV, VI:** EOMI without nystagmus or diplopia.
- CN V, VII:** Facial sensation intact; symmetrical motor function.
- CN VIII-XII:** Hearing intact; uvula midline; tongue protrudes straight.

III. Motor System & Reflexes

- Motor Strength:** 5/5 power in all extremities (Upper/Lower bilaterally).
- Bulk & Tone:** Normal muscle bulk; no atrophy, rigidity, or spasticity.
- Reflexes:** DTRs 2+ symmetric (Biceps, Triceps, Patellar, Achilles).
- Plantar:** Flexor plantar response bilaterally (Negative Babinski).

IV. Sensory, Coordination & Gait

- Sensory:** Mild decreased pinprick over palmar aspect of right digits I–III.
- Coordination:** Intact finger-to-nose and heel-to-shin bilaterally.
- Romberg:** Negative (stable balance with eyes closed).
- Gait:** Steady, narrow-based casual and tandem walk.

3. MULTI-MODAL DIAGNOSTIC TEST RESULTS

Diagnostic Test	Protocol / Scope	Key Clinical Findings	Interpretation
Brain MRI	3T Multi-planar T1, T2, FLAIR w/o contrast	Normal parenchyma. No acute infarction, hemorrhage, mass effect, or demyelinating lesions. Ventricles non-dilated.	NORMAL
Non-Contrast CT	Axial Head CT (1.25mm slices)	No intracranial hemorrhage, skull fractures, or midline shift. Normal calvarium and ventricular size.	NORMAL
Routine EEG	21-Channel Awake/Drowsy EEG (10-20)	Symmetric, reactive 10 Hz posterior dominant alpha rhythm. No epileptiform spikes or focal cerebral slowing.	NORMAL
EMG / NCS	Bilateral Motor & Sensory NCS + APB EMG	Right Median Motor Distal Latency extended at 4.6 ms (Normal <4.2 ms). CMAP amplitude 5.2 mV. No active denervation.	ABNORMAL

Axial T2 Brain MRI Slice
(Intact Parenchyma & Ventricles)

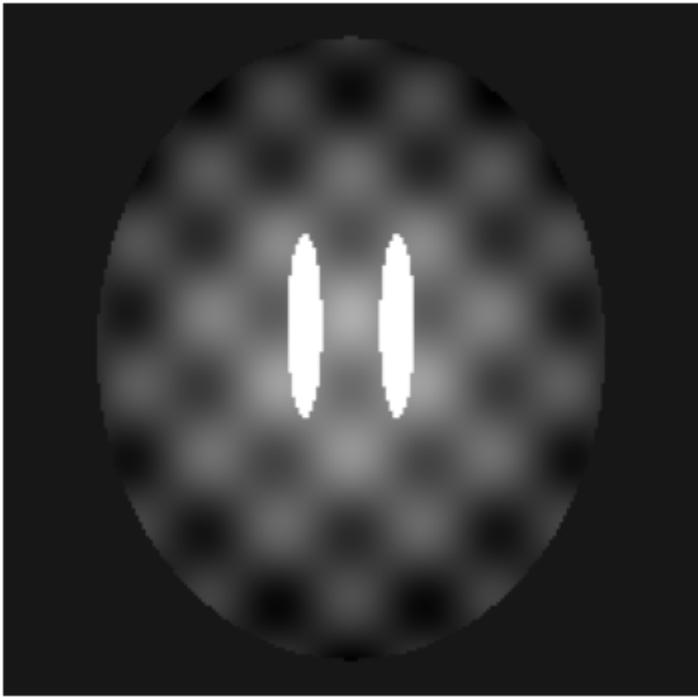


Figure 1.1: Brain MRI (Axial T2) demonstrating normal brain parenchymal signal and symmetrical ventricles.

10-Second Continuous EEG Waveforms (Normal 10Hz Alpha Background Rhythm)

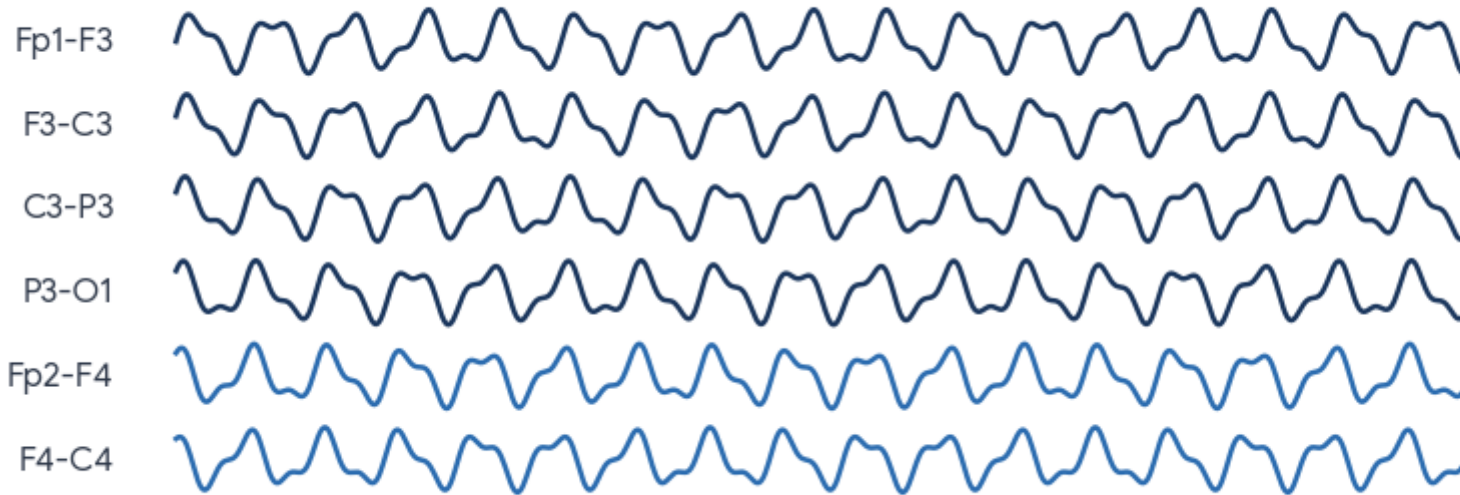


Figure 2.1: Electroencephalogram (EEG) trace demonstrating continuous, well-organized 10Hz occipital alpha rhythm.

4. CLINICAL IMPRESSION & DIAGNOSTIC SUMMARY

- 1. **Mild-to-Moderate Right Carpal Tunnel Syndrome (ICD-10: G56.01):** Electrophysiologically confirmed right median mononeuropathy at the wrist with prolonged motor distal latency (4.6 ms). Sensory examination corroborates localized median nerve distribution paresthesia.
- 2. **Normal Central Nervous System Structure & Electrophysiology:** Normal brain MRI, head CT, and routine EEG ruling out central space-occupying lesions, vascular events, or seizure disorders. Headaches are benign tension-type pattern.

5. RECOMMENDATIONS & TREATMENT PLAN

- **Conservative Management:** Neutral wrist splinting for the right hand during night time and computer use for 6 weeks.
- **Ergonomic Adjustments:** Workstation modification, frequent micro-breaks during wrist-repetitive activities.
- **Symptomatic Pharmacotherapy:** Short course of oral NSAIDs as needed for acute nerve irritation or tension headache.
- **Follow-Up:** Clinical reassessment in 8 weeks. If symptoms persist or motor weakness emerges, consider corticosteroid injection or surgical carpal tunnel release consultation.

* Electronic diagnostic consultation report validated by attending neurologist.
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